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Disparities In Access To Robotic Groin Surgery: A National Inpatient Sample Analysis

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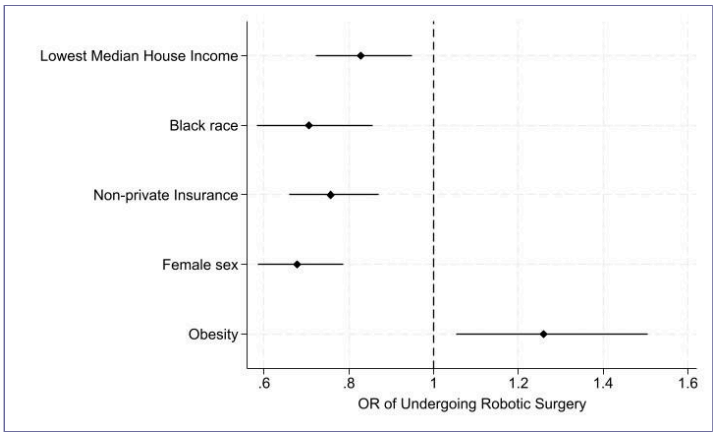
Introduction: The robotic approach has been gaining interest for its several technical and visualization advantages. The uptake of this surgical approach has also been increasing significantly. However, skepticism remains due to cost and the learning curve. Therefore, we aimed to investigate the socioeconomic and racial disparity in access to robotic groin hernia surgery.
Methods: We analyzed National Inpatient Sample from 2019 and 2021 to identify patients ≥18 years of age with inguinal or femoral hernia who underwent surgery. We examined trends in robotic utilization and disparity in access to robotic surgery based on income, insurance status, and race. Lasso regression analysis was performed to select our logistic regression model adjusting for patient's comorbidities and hospital characteristics.
Results: Of the 10,913 patients who underwent surgery for groin hernia repair, 1,551 patients (14.2%) underwent robotic surgery. Robotic surgery utilization increased between 2019 and 2021 (12.7% vs 15.9%, p=0.0001), and was more frequent in elective surgery (32.5% vs 9%, p=0.0001). Robotic approach was less likely to be considered in patients with lowest median house income (OR:0.83, 95%CI: 0.72-0.95), patients without private insurance (OR:0.76, 95%CI: 0.66-0.87), female patients, (OR:0.68, 95%CI :0.58-0.79), and black patients (OR:0.71, 95%CI: 0.58-0.86).
Conclusion: There is inequality in access to robotic surgery in groin hernia. This could be driven by limited availability or cost concerns. However, these disparities should not be ignored as technology advances in general surgery practice. Further evaluation of contributing factors should be carried out to ensure equal access.



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